

Chemotherapy and Targeted Therapy for Endocrine-Pretreated or Hormone Receptor–Negative Metastatic Breast Cancer: ASCO Guideline Rapid Recommendation Update

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ASCO Rapid Recommendations Updates highlight revisions to select ASCO guideline recommendations as a response to the emergence of new and practice-changing data. The rapid updates are supported by an evidence review and follow the guideline development processes outlined in the ASCO Guideline Methodology Manual. The goal of these articles is to disseminate updated recommendations, in a timely manner, to better inform health practitioners and the public on the best available cancer care options.

BACKGROUND

In 2021, ASCO published a guideline on chemotherapy and targeted therapy for patients with human epidermal growth factor receptor 2 (HER2)–negative metastatic breast cancer that is either endocrine-pretreated or hormone receptor–negative.¹ That guideline was updated in August 2022 to incorporate the results of the DESTINY-Breast04 trial.² The results of the TROPiCS-02³ trial, published on October 10, 2022, provided another signal to update.

METHODS

A targeted electronic literature search was conducted to identify any additional phase III randomized controlled trials of treatment options in this patient population. No additional randomized controlled trials were identified. The original guideline Expert Panel was reconvened to review new evidence from TROPiCS-02³ and to review and approve the revised recommendation.

EVIDENCE REVIEW

TROPiCS-02³ was an international, randomized, phase III trial that compared sacituzumab govitecan (SG) (n = 272) against four other chemotherapy options (single-agent eribulin, vinorelbine, capecitabine, or gemcitabine), which comprised treatment of physician's choice (TPC) (n = 271) in 543 patients with endocrine-resistant hormone receptor–positive and HER2-negative locally recurrent inoperable or metastatic breast cancer who had received 2–4 prior chemotherapy regimens for metastatic disease. The primary end point for TROPiCS-02 was progression-free survival (PFS) as assessed by blinded independent central review.

The PFS primary end point at the first planned interim analysis was presented at ASCO 2022 and recently

published in the *Journal of Clinical Oncology*. After a median follow-up of 10.2 months, the primary end point was met with a 34% reduction in risk of progression or death (hazard ratio [HR], 0.66; 95% CI, 0.53 to 0.83; $P = .0003$). The median PFS was 5.5 months (95% CI, 4.2 to 7.0) with SG and 4.0 months (95% CI, 3.1 to 4.4) with TPC. At 6 months, the PFS was 46% (95% CI, 39 to 53) versus 30% (95% CI, 24 to 37), and at 12 months, it was 21% (95% CI, 15 to 28) versus 7% (95% CI, 3 to 14). At that time, median overall survival was not yet mature (HR, 0.84; $P = .14$). Key grade ≥ 3 treatment-related adverse events with SG were diarrhea (5%) and neutropenic complications including febrile neutropenia (4%) and neutropenic colitis (2%).

At the planned second interim analysis after a median follow-up of 12.5 months, SG had a statistically significant improvement in overall survival compared with TPC (HR, 0.79; 95% CI, 0.65 to 0.96; $P = .020$).⁴ The median overall survival was 14.4 versus 11.2 months in patients treated with SG and TPC, respectively. Appraisal of the trial report using the GRADE⁵ instrument was performed as per ASCO's methodology and found a moderate certainty of the evidence.

UPDATED RECOMMENDATION

Patients with hormone receptor–positive HER2-negative metastatic breast cancer who are refractory to endocrine therapy and have received at least two prior lines of chemotherapy for metastatic disease may be offered SG. (Type: Evidence-based, benefits outweigh harms; Evidence quality: Moderate; Strength of recommendation: Strong.)

For all guideline recommendations, a complete summary table is available at www.asco.org/breast-cancer-guidelines.

ASSOCIATED CONTENT

The companion to this article was published in the December 10, 2021 issue of *Journal of Clinical Oncology*. See accompanying article on page 3938

Author affiliations and support information (if applicable) appear at the end of this article.

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Guideline and Conflicts of Interest

The Expert Panel was assembled in accordance with ASCO's Conflict of Interest Policy Implementation for Clinical Practice Guidelines ("Policy," found at <https://www.asco.org/guideline-methodology>). All members of the Expert Panel completed ASCO's disclosure form, which requires disclosure of financial and other interests, including relationships with commercial entities that are reasonably likely to experience direct regulatory or commercial impact as a result of promulgation of the guideline. Categories for disclosure include employment; leadership; stock or other ownership; honoraria, consulting or advisory role; speaker's bureau; research funding; patents, royalties, other intellectual property; expert testimony; travel, accommodations, expenses; and other relationships. In accordance with the Policy, the majority of the members of the Expert Panel did not disclose any relationships constituting a conflict under the Policy.

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EDITOR'S NOTE

This ASCO Clinical Practice Guideline Recommendation Update provides a recommendation update, with review and analysis of the relevant literature for the recommendation. Additional information, including links to patient information at www.cancer.net, is available at www.asco.org/breast-cancer-guidelines.

EQUAL CONTRIBUTION

B.M. and L.A.C. were Expert Panel coauthors.

AUTHORS' DISCLOSURES OF POTENTIAL CONFLICTS OF INTEREST

Disclosures provided by the authors are available with this article at DOI <https://doi.org/10.1200/JCO.22.02807>.

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Conception and design: All authors

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Collection and assembly of data: All authors

Data analysis and interpretation: All authors

Manuscript writing: All authors

Final approval of manuscript: All authors

Accountable for all aspects of the work: All authors

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